RIVM Public Health Foresight Study: Trendscenario 2022–2050

National Institute for Public Health and the Environment (RIVM) & Statistics Netherlands (CBS) • SAS Replication Analysis
^S={font_face='Arial' font_size=8.5pt italic color=#71717a}Source: https://www.volksgezondheidtoekomstverkenning.nl/vtv-2024/trendscenario • Projections through 2050

Executive Summary: Master Findings & Strategic Projections (2022 vs. 2050)

Domain	Indicator / Horizon	Baseline (2022)	Projected (2050)
Levensverwachting	Levensverwachting	Levensverwachtin	Gezonde
Levensverwachting	Levensverwachting Mann	g Le	Levensjaren
Overleving Kans op	Overleving Kans op leeftijd 100	Chronische	Chronische Ziekten
leeftijd 85 bere	bereiken	Ziekten P	M

Trend Dynamic & Epidemiological Context
Gezonde
Top

^S={font_face='Arial' font_size=8pt italic color=#71717a}
Note: Trendscenario assumes continuation of existing policies and historical trends without major systemic policy interventions.

Pillar 1: Life Expectancy Trajectory & Longevity Frontier (1990–2050)

Figure 1: Observed and projected life expectancy at birth in the Netherlands for Total, Men, and Women.

^S={font_face='Arial' font_size=8pt italic color=#71717a}Figure 1 Note: Total life expectancy rises from 81.6 years in 2022 to 86.5 years in 2050. The female advantage remains stable at 3.4 years. Linked to Table 1.

Table 1: Life Expectancy and Healthy Longevity Milestones (Selected Years)

Year	Total LE	Men LE	Women LE	Healthy LE (Good Health)	Disability-Free LE
18181	735.6	718.1	752.8	582.9	642.6

Pillar 2: Cumulative Survival Probability to Milestone Ages (1990–2050)

^S={font_face='Arial' font_size=8pt italic color=#71717a}Figure 2 Note: The chance of reaching age 85 increases from 50.3% to 68.9%. The chance of becoming a centenarian (100+) expands from 1.3% to 4.2% (over 3x increase).

Pillar 3: Chronic Disease Population & Age Stratification (2022–2050)

Figure 3: Growth in individuals living with chronic illness by age cohort.

^S={font_face='Arial' font_size=8pt italic color=#71717a}Figure 3 Note: Population with chronic illness increases by 1.5 million to 12.0 million in 2050, with 87% (+1.3 million) concentrated among individuals aged 75 and older. Linked to Table 2.

Pillar 4: Top Chronic Conditions in 2050 (Baseline vs. Projected Prevalence)

^S={font_face='Arial' font_size=8pt italic color=#71717a}Figure 4 Note: Artrose (osteoarthritis) surges to become the #1 chronic condition in the Netherlands with nearly 3.0 million patients in 2050 (+78.1% growth). Linked to Table 3.

Table 3: Top Chronic Conditions Ranked by 2050 Prevalence and Growth Rate

Chronic Condition	2022 Patients	2050 Patients	Absolute Growth	Growth (%)
Artrose (Osteoarthritis) 1671900 2977800	.	.	.	.
Gezichtsstoornissen (Vision) 1003900 140	.	.	.	.
Diabetes Mellitus 1166600 1246600	.	.	.	.
Gehoorstoornissen (Hearing) 800100 11335	.	.	.	.
Schildklierziekten 677700 821400	.	483,300	.	.
Coronaire hartziekten 795800 697800	.	879,000	.	.

Pillar 5: National Disease Burden Shift (DALYs: 2022 vs. 2050)

Figure 5: Total Disability-Adjusted Life Years (DALY = YLL + YLD) by primary condition.

^S={font_face='Arial' font_size=8pt italic color=#71717a}Figure 5 Note: Dementia burden increases by +146.1% to 346,300 DALYs, surpassing mood disorders to become the single largest source of disease burden in the Netherlands. Linked to Table 4.

Pillar 6: Historic Transition in Causes of Death (Mortality Shift)

^S={font_face='Arial' font_size=8pt italic color=#71717a}Figure 6 Note: Cardiovascular disease mortality drops from #2 (39,000) to #6 (18,900) due to prevention and interventions, while dementia/psychiatric mortality rises to #2 (36,000). Linked to Table 5.

Table 5: Primary Cause of Death Transitions and Rank Trajectories (2022 vs. 2050)

Rank '22	Rank '50	Cause of Death Classification	2022 Deaths	2050 Deaths	Net Change
.	**13000**	Nieuwvormingen (Cancers) 47500 47100 1 1	.	.	.
.	**10500**	Ziekten ademhalingswegen (Resp) 12900 24800 4	.	.	.
.	**.**	Zenuwstelsel en zintuigen 9100 20000 7 5	.	.	.
3800	**8500**	Symptomen & onvolledig omschreven 10600 8600	.	.	.

Pillar 7: Socioeconomic Inequality in Life Expectancy & Health Gaps

Figure 7: Total Life Expectancy vs. Healthy Life Expectancy by educational attainment.

^S={font_face='Arial' font_size=8pt italic color=#71717a}Figure 7 Note: While the total life expectancy gap between low and high education is 4.9 years, the gap in Healthy Life Expectancy reaches 14.6 years (53.6 vs. 68.2 years). Linked to Table 6.

Table 6: Educational Disparities in Longevity and Perceived Health Horizon

Education Group	Health Longevity Metric	Expected Years
Laag	Levensverwachting	78.4
Middelbaar	Levensverwachting	81.1
Hoog	Levensverwachting	83.3
Laag	Gezonde_Levensverwachting	53.6
Middelbaar	Gezonde_Levensverwachting	61.2
Hoog	Gezonde_Levensverwachting	68.2

Strategic Conclusions for Dutch Healthcare & Prevention Policy:

1. Longevity Expansion: Total LE reaches 86.5 years, but 1.2 years of the gain will be lived with functional disability.
2. Multimorbidity Pressure: Over 4.3 million citizens will have 3 or more chronic conditions by 2050, necessitating integrated generalist care.
3. The Dementia Tsunami: With DALYs surging +146% to 346,300, dementia becomes the #1 national healthcare challenge.
4. Persistent Inequalities: Targeted prevention must address the 14.6-year healthy life expectancy gap across socioeconomic gradients.